

CLINICAL TRIAL PROTOCOL

A Phase III, Randomized, Double-Blind, Placebo-Controlled Study to Evaluate the Efficacy and Safety of NeuroX in Patients with Early-Stage Alzheimer's Disease

Protocol Identifier: PRO-2026-NEURO-042

IND Number: IND-168420

NCT Number: NCT09876543

Sponsor: Nexus Therapeutics Inc.

Investigational Product: NeuroX (10mg and 20mg Tablets)

Development Phase: Phase III

Date of Protocol: March 2026

Version: 2.0 (Detailed Structural Framework)

CONFIDENTIALITY STATEMENT

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1. STUDY COHORTS & DESIGN STRATIFICATION

To evaluate dose-dependent efficacy, safety, and biomarker dynamics, the target population of 450 subjects is allocated into three distinct study cohorts at Randomization (Visit 2) using a 1:1:1 stratification ratio.

- **Cohort A (Experimental High Dose):** Target enrollment of 150 subjects. Participants receive active NeuroX 20mg orally once daily for 24 consecutive weeks.
- **Cohort B (Experimental Low Dose):** Target enrollment of 150 subjects. Participants receive active NeuroX 10mg orally once daily for 24 consecutive weeks.
- **Cohort C (Placebo Control):** Target enrollment of 150 subjects. Participants receive matching inert placebo tablets orally once daily for 24 consecutive weeks.

1.1 Randomization Strata

Randomization will be performed via an Interactive Response Technology (IRT) system, stratified based on baseline MMSE score severity (20-22 vs. 23-26) and concurrent use of stable background acetylcholinesterase inhibitors (Yes vs. No).

2. DETAILED STUDY VISITS

The trial comprises four protocol-defined clinic visits and one remote safety follow-up interface.

Visit 1: Screening (Window: Day -14 to Day -1)

Establishes study eligibility via definitive NIA-AA diagnostics, cognitive thresholds (MMSE 20-26), extensive medical history validation, safety ECG, physical examinations, and comprehensive baseline hematology and chemistry panel draws.

Visit 2: Baseline & Randomization (Window: Day 1)

Confirmation of continuing eligibility criteria. Completion of the primary cognitive efficacy measure (ADAS-Cog 13). IRT initialization for cohort assignment and initial Interactive Response Technology drug kit dispensing.

Visit 3: Midpoint Evaluation (Window: Week 12 $\pm$ 3 Days)

Safety monitoring milestone evaluating treatment-emergent adverse events (TEAEs), vital signs tracking, patient compliance checks via container weight logs, repeat safety blood panels, and midpoint ADAS-Cog 13 evaluation.

Visit 4: End of Treatment (EOT) (Window: Week 24 $\pm$ 5 Days)

Final on-treatment efficacy and safety assessments. Includes exit physical checkups, definitive ADAS-Cog 13 measurement, secondary functional scale checks, final biofluid biomarker draws (A β 42 and pTau-181), and full drug accountability / kit return processing.

Visit 5: Safety Follow-up (Window: Week 28 $\pm$ 7 Days)

A remote telephone safety check to document any persistent or newly emergent delayed adverse experiences, updates to concomitant therapeutic regimens, or hospitalization incidents post-treatment discontinuation.

3. INVESTIGATIONAL KITS & SUPPLY CHAIN MANAGEMENT

All investigational items are packaged, tracked, and barcoded within unique, multi-layered block kits to preserve double-blind integrity across clinical centers.

KIT-IP-20MG: NeuroX 20mg active tablet configuration. Contains a 30-day bottle supply (35 tablets including overage buffers). Double-blind labeled. Store at controlled room temperature 20°C to 25°C.

KIT-IP-10MG: NeuroX 10mg active tablet configuration. Contains a 30-day bottle supply (35 tablets including overage buffers). Double-blind labeled. Store at controlled room temperature 20°C to 25°C.

KIT-PBO-00: Matching placebo configuration. Identical physical layout, weight, color, odor, and taste profile to active tablets. Double-blind labeled. Store at controlled room temperature 20°C to 25°C.

KIT-LAB-CORE: Central Laboratory Collection Block. Pre-packaged ambient box containing color-coded vacutainers (EDTA, Serum separators), tracking manifests, and chilled shipping containers for hematology, biochemistry, and biofluid biomarker processing.

3.1 Dispensation and Accountability Logic

At Visits 2 and 3, investigators must enter the subject ID directly into the web-based IRT module. The IRT algorithm matches the subject's randomized cohort assignment and yields a specific, validated kit barcode sequence available in local pharmacy inventory. Physical kit hand-off cannot occur without electronic authorization logs.

4. SCHEDULE OF EVENTS MATRIX

The grid below cross-references all mandatory procedural interactions, laboratory evaluations, and investigational kit handoffs required at each protocol-defined time point.

Core Procedure / Evaluation	V1: Screening Days -14 to -1	V2: Baseline Day 1	V3: Midpoint Week 12	V4: EOT Week 24	V5: Safety F/U Week 28 (Remote)
Informed Consent & Screening ID Log	X				
Medical History & NIA-AA Review	X				
Inclusion/Exclusion Verification	X	X			
Physical Exam & Vital Signs Tracking	X	X	X	X	
Cognitive Baseline Screening (MMSE)	X				
Primary Efficacy Scale (ADAS-Cog 13)		X	X	X	
Safety Hematology & Chemistry Panels	X		X	X	
Specialized Alzheimer Biomarkers (CSF)		X		X	
Adverse Event & Con-Med Reconciliation	X	X	X	X	X
IRT Allocation & Randomization Script		X			
Core Lab Assembly Dispatched (KIT-LAB-CORE)	X	X	X	X	
Investigational Kit Dispensing via IRT		X	X		
Drug Kit Return & Full Accountability Logs			X	X	

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